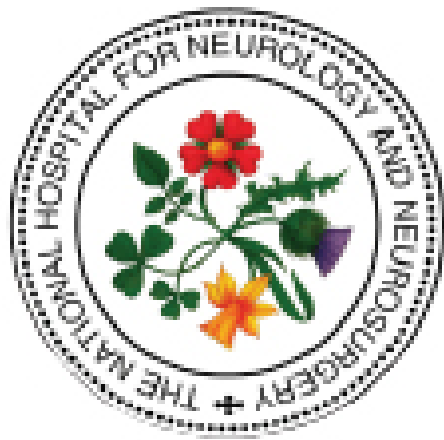




The National Hospital for Neurology and Neurosurgery Neuroscience Critical Care Unit (NCCU) **ORGAN RETRIEVAL PATHWAY at NHNN**

Wherever possible organ retrieval at NHNN should take place outside of normal working hours, or during emergency theatre availability

There are only provisions for emergency surgery during the morning at NHNN, with one theatre available

Emergency cases in other working hours 'interrupt' scheduled operating lists and are allocated to the theatre that will minimise disruption to elective surgery. This generally works well because most of the emergency surgery is of short duration.

Organ retrieval is dealt with in a similar manner but, because it is a lengthy procedure lasting several hours, it risks stopping whole elective lists if performed during normal working hours.

FOR ALL CASES:

- A clear timeline for organ retrieval should be agreed with local NCC and theatre staff by the SNOD at the earliest opportunity
- Deviations from this timeline should be discussed with local staff (particularly theatre manager or deputy and NCC nurse in charge) and any impact on other services agreed jointly

PERSONNEL INVOLVED IN DECISIONS TO CANCEL ELECTIVE SURGERY OR DELAY DONATION

- Decisions to cancel surgery to allow organ retrieval to take place or to delay donation until theatre team becomes free, must be made on case-by-case basis by MDT. This must include, but is not restricted to:
- SNOD
 - NCC nurse in charge
 - NCC consultant
 - Theatre manager or deputy
 - Anaesthetic coordinator in working hours
 - NHSBT Regional manager on call
 - Consultant neurosurgeon
 - The donor's family if delayed retrieval is being considered

MAXIMISING CHANCE of ORGAN RETRIEVAL TAKING PLACE OUTSIDE NORMAL WORKING HOURS

1. Brainstem death testing should be undertaken, and decisions to withdraw made, early in the day whenever possible
2. Samples for virology and tissue typing should be dispatched in a timely manner
3. If the donor has cardiothoracic potential, the SCOUT should be mobilised soon after consent
4. Offering of organs should proceed in a timely manner, for cardiothoracic organs in particular - *It must be noted that cardiothoracic offering can be a lengthy process in itself*
5. Retrieval teams must arrive at NHNN within the three-hour window stipulated in national guidance - *Where this does not occur the delay must be escalated to the NHSBT Team Manager in working hours or the NHSBT Regional Manager out of hours, and a second team mobilised*

MINIMISING DISRUPTION IF 'IN HOURS' RETRIEVAL REQUIRED

1. To minimise disruption to elective operating lists, a cardiothoracic organ retrieval should commence (i.e. the first incision should be made) by 05:30am and an abdominal retrieval by 06:30am. If these start times are not achievable and organ retrieval will encroach significantly into routine operating time, the following issues should be considered
2. Is there a spare theatre available or can lists be re-arranged to make a theatre free?
3. Is there ICU bed/nurse availability to allow the retrieval to be postponed to the end of the working day (if the donor is stable)?
4. Can donation be delayed for a short time to allow some urgent elective cases to be carried out first? This will also depend on (2) above
5. Do the family support a delayed retrieval?